

CDSS Sample Protocol Upload

Protocol ID: ENDO-DM-CKD-2026-v2

Title: Integrated Outpatient Management Protocol for Type 2 Diabetes with CKD Stage 3

Service Line: Endocrinology and Nephrology Co-Management

Effective Date: 2026-01-15

Review Cycle: Every 12 months

Clinical Scope:

- Adults age 18 years or older with type 2 diabetes and CKD stage 3 (eGFR 30–59 mL/min/1.73m²).
- Applies to ambulatory care and post-discharge follow-up visits.
- Excludes pregnancy, dialysis dependence, and kidney transplant recipients.

Baseline Evaluation (within 14 days of enrollment):

1. Confirm CKD stage using two eGFR values at least 90 days apart.
2. Obtain urine albumin-creatinine ratio (UACR), basic metabolic panel, HbA1c, and CBC.
3. Reconcile medication list, focusing on NSAIDs, RAAS agents, and glucose-lowering therapy.
4. Document blood pressure, weight, BMI, smoking status, and volume status.

Primary Treatment Recommendations:

1. Start or continue an SGLT2 inhibitor if eGFR threshold and tolerability criteria are met.
2. Use ACE inhibitor or ARB for albuminuric CKD unless contraindicated.
3. Individualize glycemic target, generally HbA1c around 7.0–7.5%, with less stringent goals for high hypoglycemia risk.
4. Target blood pressure below 130/80 mmHg when tolerated.
5. Add statin therapy for cardiovascular risk reduction unless contraindicated.

Medication Safety Rules:

- Avoid routine NSAID use because of AKI and CKD progression risk.
- During SGLT2 inhibitor initiation, assess orthostatic symptoms and reduce loop or thiazide dose if needed.
- If recurrent hypoglycemia occurs, de-intensify insulin or sulfonylurea before stopping kidney-protective therapies.
- Review potassium and creatinine 1–2 weeks after RAAS dose changes.

Monitoring Cadence:

- eGFR and creatinine: every 3 months for stable CKD stage 3, sooner after medication changes.
- UACR: every 3–6 months.
- HbA1c: every 3 months if uncontrolled, every 6 months if stable.
- Blood pressure and weight: every visit.

Escalation Triggers:

- eGFR decline more than 5 mL/min/1.73m² per year.
- UACR increase more than 30% despite adherence.
- Persistent potassium above 5.5 mmol/L.
- Recurrent AKI, severe edema, or resistant hypertension.

Referral Criteria:

- Nephrology referral for rapid decline, eGFR below 30, or difficult electrolyte management.
- Pharmacist referral for polypharmacy and drug interaction risk.
- Dietitian referral for sodium, protein, and carbohydrate planning.

Expected Outcomes at 6 Months:

- Slower annualized eGFR decline.
- Lower albuminuria trend.
- Improved blood pressure control and reduced unplanned renal-related visits.

Evidence Basis:

- Strong evidence for SGLT2 use and RAAS blockade in albuminuric CKD.
- Moderate evidence for structured multidisciplinary follow-up reducing avoidable deterioration.
- Consensus-backed recommendations for stepwise medication safety monitoring.

Operational Notes:

- This is sample protocol content for testing ingestion, retrieval, filtering, and citation workflows.
- Not a substitute for local clinical governance policies.